

THE 7TH TIMEPIE LONGEVITY FORUM · BIOHACKER TRACK

BIOHACK.IT



Turn self-experimentation into evidence.

Open infrastructure for structured human self-experimentation.

PRESENTED AT



September 12–13, 2026 · Shanghai

Open source · AGPL-3.0 · Non-profit — Hacking Biology

THE GAP

Longevity intervention is running far ahead of longevity measurement.

People already adopt off-label drugs, peptides, HBOT, plasmapheresis. But the protocol lives in spreadsheets, forum posts and chat prompts — not comparable, not followable, not readable by a physician.

130+

biomarkers a serious protocol tracks —
blood, DNA methylation, DEXA

$N = 1$

an isolated self-experiment carries almost
no statistical power

0

shared representation layer for protocol &
biomarker across people and labs

Bryan Johnson has a lab and a team. Everyone else has a spreadsheet. A thousand N-of-1 on the same protocol, made comparable, are a different scientific object — and it doesn't exist yet.

WHO IS EXPOSED

The beginner who copies is the person most at risk.

PRIMARY USER

The experienced biohacker

Designs, self-administers, iterates — but the results today are just anecdotes: claimed, not measured, impossible to compare.

HIGHEST RISK

The beginner who copies

Reads a forum thread or an influencer hyping a compound and starts taking it — no baseline, no safety markers, no measurement.

RECIPIENT, NOT USER

The treating physician

Today receives, at best, a handwritten sheet. Deserves a legible, complete view of what is taken and why.

The person who copies an off-label compound off a forum thread — without knowing to monitor kidney and liver — is exactly the case where the platform protects health.

WHAT MAKES IT COMPUTABLE

The same protocol, written three ways, is one entity.

This is the whole project in three lines. Our real asset is not an app — it is standardising something nobody has standardised.

"rapamycin 6mg weekly"
"sirolimus 6 mg every Sunday"
"Rapa 6 mg q7d"

human-readable, research-useless



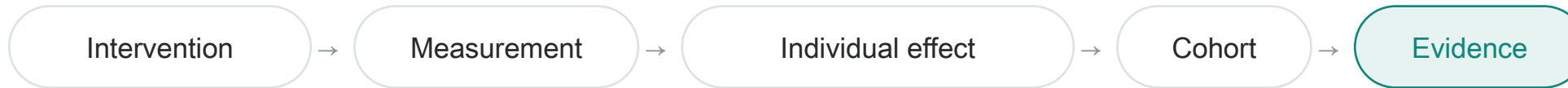
RxNorm 35302 · 6 mg · every 7 days · pulsed

one computable entity, linked to your biomarkers over time

Do this ten thousand times and a forum's worth of prose becomes a dataset.

HOW IT WORKS

From one intervention to shared evidence.



- ◆ **Intervention** what you take and do — dose, timing, real cycle schema, versioned with every change.
- ◆ **Measurement** standards-coded biomarkers (LOINC/UCUM) with lab, method and provenance attached.
- ◆ **Individual effect** your own before/after, bound to the protocol version that was active at the time.
- ◆ **Cohort** everyone running the same protocol, weighted by adherence and data completeness.
- ◆ **Evidence** an open dataset the community — and researchers — can actually interrogate.

THE PLATFORM

Document your protocol. Measure it. Share it.

01 — DOCUMENT

What you take & do

Substances, therapies, exercise, nutrition — with dose, timing and real cycle schemas. Create from scratch, or copy any public protocol with one click.

02 — MEASURE

Efficacy & safety

Biomarkers with reference, optimal and safety ranges, plus biological clocks (PhenoAge, epigenetic, glycation). Upload past lab reports, get your history in graphs.

03 — SHARE

Open by default

Publish your protocol and its outcomes. Quality comes from three layers: deterministic validation, community review, expert oversight.

Open source, free, non-profit — built on the community that already exists, not a forum rebuilt from scratch.

FOR HACKERS

GitHub for biological self-experimentation.

If you have ever used version control, you already know how this works.

repository

a protocol

issues

a contested claim, discussed in public

fork

copying someone's protocol

telemetry

your biomarkers over time

commit

a dose change, with its reason

README

the doctor sheet

release

a protocol version

dataset

the open research output

CI

safety monitoring — baseline, inherited
markers, overdue

DATA MODEL

Three questions, deliberately kept apart.

TestingProtocol

What I measure, and when. A versioned set of measures and timepoints.

TreatmentPlan

What I take and do. Versioned interventions — self-managed or physician-assigned.

Goal

What I want to change. Mapped onto the Hallmarks of Aging.

Substance → SafetyRule → Biomarker → Measurement → Dashboard

“What you take” and “how you are” are the same data structure, read from two sides.

HARM REDUCTION

Follow a protocol — inherit its safety.

- ◆ **Mandatory baseline** before you can follow a protocol at all — plus an explicit risk acknowledgment.
- ◆ **Safety markers inherited automatically** from every active compound — safety is never opt-in.
- ◆ **Dose sanity check** catches the extra zero — plausibility validation of a number, not clinical advice.
- ◆ **Overdue is a safety gap** “rapamycin active for 94 days, liver panel overdue by 34.”

This is the module that justifies the project. Biohackers already take the risk today — without any of it.

THE MVP · WHAT SHIPS FIRST

Upload your past lab reports. Get your clinical history in graphs. Free.

A HARD BOUNDARY

The LLM writes words, never numbers

Extraction, mapping and thresholds are deterministic and reproducible. The model only produces the plain-language narration — generated once and cached.

BUILT FOR AGGREGATION

LOINC + UCUM + method, every value

The only architecture compatible with releasing open data. Three ranges per analyte: laboratory reference, longevity-optimal, safety — each with its source.

A sentence you can say on television without pronouncing the word “biohacking”, with no regulatory exposure — and it works with a single user and no community.

WHAT THIS IS — AND WHAT IT IS NOT

We are not a clinical trial, and we say so first.

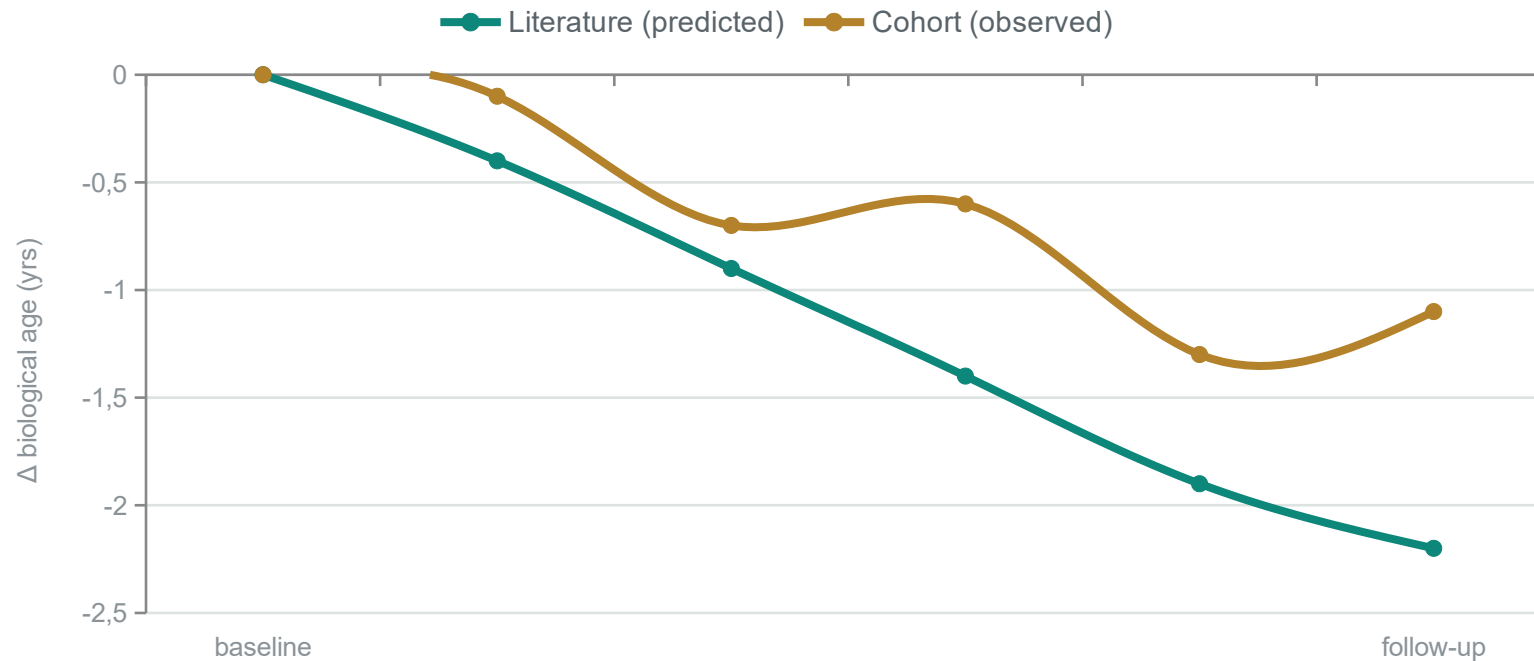
This is self-managed, self-declared, self-selected experimentation. No randomisation, no blinding, no controlled conditions. People change protocol halfway, measure irregularly, and drop out.

The honest comparison is not biohack.it versus a clinical trial — we lose that one, and we are not trying to win it.
It is biohack.it versus what exists today: a dose written in prose, a screenshot of a lab report, a spreadsheet nobody else can open.

- ◆ **Every number carries its confidence** value, uncertainty, provenance and evidence level — or it is not shown.
- ◆ **Observed, never caused** “among qualifying users exposed to X, the observed change was Y.”
- ◆ **Negative results are welcome** “I tried X. Nothing happened.” — the data biohacking always loses.

WHAT NO ONE ELSE CAN BUILD

What the literature predicts, beside what practitioners actually show.



THE ORIGINALITY

**Outcome ↔
Biomarker**

Anyone can draw the first line — it is in the literature. Only a platform with a living community and shared, standards-coded biomarkers can draw the second, and put them side by side.

Illustrative. We report what was observed in a self-selected group — never that the intervention caused it.

OPEN BY DEFAULT

Open data first, linked data second, API third.

SELF-GENERATED

Open & linked data

Everything public — profiles, protocols, treatments, measurements — is released as a clonable snapshot, coded and linked, sufficient to seed an independent instance.

THEN

Research API

A documented endpoint returning de-identified, methodologically annotated cohort data. Ask for an intervention and a biomarker, get an answer with its caveats attached.

BY CONSTRUCTION

Impossible to enclose

AGPL-3.0: anyone running it as a service must open the source. Fork it, self-host it, keep the community's data in the community's hands.

Standards, not silos: HL7 FHIR internally, OMOP for research export, LOINC/UCUM for analytes — so what you produce is research data, not another wellness silo.

BUILT IN THE OPEN

The whole specification is public before the first line of product.

24 capabilities, 140+ requirements — protocols, biomarkers, safety, community, evidence and open data — browsable at biohack.it/specs, growing with community review.

Protocols

create · copy · import · versioned

Blood Layer

biomarkers · clocks · deterministic

Safety guardrails

baseline · inheritance · dose check

Community & Studies

follow/copy · cohorts · N-of-1

Open Data & Standards

LOINC/UCUM · FHIR/OMOP · clonable

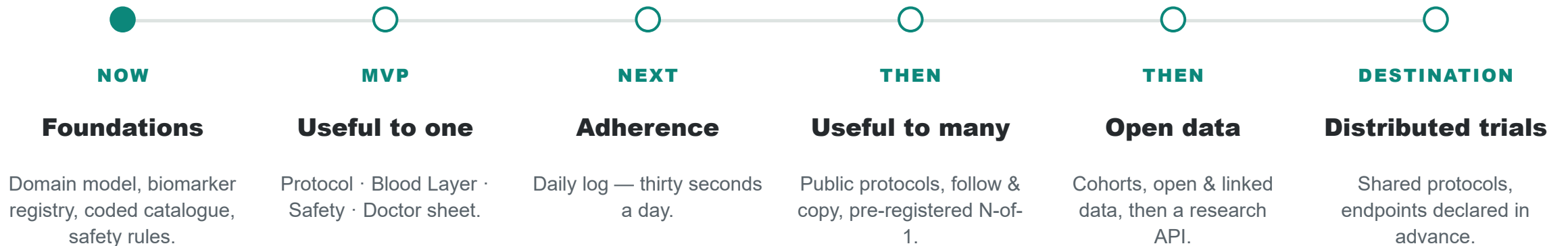
AI, Evidence & Provenance

AI4L · Evipedia · attribution

Specifying broadly costs little. Building broadly costs everything — so what we build first is deliberately narrow.

MVP FIRST

Breadth in thinking, discipline in shipping.



Deferred without regret: procurement, advanced nutrition, therapeutics scheduling, genomics beyond import, agent access.

JOIN US

Build this with us.

This is open infrastructure, and it is early. Every one of these roles is genuinely open — and each one changes what the project becomes.

Biohackers

Bring your experience into the specification, and your protocols and measurements into the software.

Software hackers

Build it. AGPL, public specification, open issues — written down before it is written in code.

Researchers & scientists

Keep us honest on validity, uncertainty and evidence — while keeping it usable by a beginner.

Longevity enthusiasts

Volunteer to spread it: community, translation, events. A tool nobody knows about produces no data.

Fundraisers & philanthropy

Build the funding pipeline — grants and philanthropy — so it never has to become commercial.

Tell us which of the five you are — and what you would like to work on.

GET IN TOUCH

Contacts & references.

Telegram Group Chat



Scan or search the channel —
project updates and open discussion.

WeChat 微信



Scan to add Fabio — he will add you
to the biohack.it group chat.

EMAIL

research@hackingbiology.com

WEBSITE

biohack.it

SOURCE

github.com/hackingbiology/biohackit

COMMUNITY

rapamycin.news



THE INVITATION



Everyone buys anti-aging. **Almost no one measures it.**

Let's measure it — together, in the open.

Open infrastructure for structured human self-experimentation — fork it, self-host it, own your data.
The community already exists; now it gets its instrument.

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AN OPEN LABORATORY FOR LONGEVITY · NON-PROFIT · AGPL-3.0